National Cancer Control Programme (NCCP) 00206 · clinician draft

Trastuzumab emtansine (T-DM1) · metastatic

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Metastatic T-DM1

Active medicines: Trastuzumab emtansine (T-DM1)

Emetogenic risk: Low

Medicine	Dose / route	When
Ondansetron	8 mg orally	On treatment day Review QT, constipation and hepatic dosing; reconcile any steroid already prescribed
Metoclopramide	10 mg orally up to three times daily as needed	Subsequent days: duration to be confirmed Assess contraindications, renal function and interactions

Additional phase-specific support

- T-DM1 has low emetogenic risk; review the low-risk antiemetic draft and individual contraindications.
- First infusion over 90 minutes, then observe for at least 90 minutes. Later infusions may take 30 minutes if tolerated, with at least 30 minutes observation.
- Contact the unit to rearrange a missed dose; the next schedule preserves a 3-week interval.
- Companion chemotherapy or endocrine drugs need their own supportive review; this draft covers the named HER2 component only.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

National Cancer Control Programme (NCCP) 00206 v5 · p4 · NCCP antiemetics v8, Table 1 pp4–5

Sources checked 2026-10-21 · SACTCheck v0.84.14 · Regimen-level drafts; prescriber and oncology-pharmacy verification required